
CPT 92229 Reimbursement Brief for Tribal Health Programs and Indian Health Service Facilities

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Audience: Tribal Health Program billing offices, IHS Area Office contacts, Tribal 638 facility administrators, Urban Indian Organization billing teams, and medical directors evaluating autonomous AI diabetic retinopathy screening for AI/AN patient populations

How to Use This Brief

This is a partner-ready document. Proactive FQHC is sharing it with Tribal Health Programs and IHS-affiliated facilities evaluating CPT 92229 deployment. It is written for your billing office, IHS Area Office contact, or medical director, not for Proactive itself.

Wherever you see "your facility" or "your Tribal Health Program" in this document, read it as your specific clinic being evaluated. The All-Inclusive Rate, billing mechanics, and grant terms are facility-specific and must be confirmed by your own billing team with your IHS Area Office and state Medicaid office.

This brief is informational. Nothing in it is a contractual commitment, and final reimbursement determinations rest with your IHS Area Office, fiscal intermediary, state Medicaid program, and any applicable grant administrator.

Scope and Caveat

This brief covers CY 2025 and CY 2026 reimbursement guidance for CPT 92229 (autonomous AI diabetic retinopathy screening) as it applies to Tribal Health Programs and IHS facilities. Sources include the Indian Health Service, the Federal Register, CMS, and applicable sections of US Code (42 USC 1396d(b) and 42 USC 1395qq).

Each partner Tribal Health Program must confirm final numbers with its own IHS Area Office and state Medicaid office before treating any figure as a commitment. AIR rates are set annually and published in the Federal Register. Billing mechanics differ across IHS-direct, Tribal 638, and Urban Indian Organization (UIO) facilities. Nothing in this brief overrides your IHS Area Office or fiscal intermediary's guidance.

Short Answer

For Tribal Health Programs and IHS-affiliated facilities, CPT 92229 is not separately payable under the All-Inclusive Rate (AIR). It is absorbed into the per-encounter AIR payment, the same way diagnostic imaging, lab work, and other services performed on the same day as a qualifying encounter are absorbed. The financial case for 92229 in a Tribal setting is not a per-screen fee. It is three things stacked together:

1. **Encounter capture at the AIR.** Every AI/AN diabetic patient who would have been referred out for retinal screening is now an in-clinic encounter that bills at the AIR. CY 2025 outpatient AIR (Lower 48): \$718 for Medicare Part B encounters, \$801 for non-Medicare (Medicaid and other) encounters. CY 2026 outpatient AIR (Lower 48): \$826 for Medicare, with Alaska rates higher (CY 2025 Alaska outpatient Medicare \$1,193, CY 2026 \$1,222). All rates are published in the Federal Register annually.
2. **GPRA Diabetic Retinopathy Screening Rate performance.** A Government Performance and Results Act (GPRA) clinical performance measure that Tribal and IHS facilities report against annually. FY 2025 national IHS DR screening target rate: 47.6% (FY 2024 was 44.7%).
3. **Special Diabetes Program for Indians (SDPI) grant protection.** SDPI was reauthorized through December 31, 2026 at \$200 million per year (up from \$160 million; signed February 2026). DR screening is one of the published SDPI Best Practices, and DR screening rate is commonly built into SDPI grant scope of work where applicable.

The rest of this brief works through each component in detail.

Who This Brief Covers

- **IHS-operated facilities** billing under the AIR.
 - **Tribal 638 facilities** (Tribes and Tribal organizations operating IHS programs under Public Law 93-638 self-determination contracts or compacts).
 - **Urban Indian Organizations (UIOs)** that have elected AIR billing.
 - **Tribal Health Programs with FQHC-Look-Alike status** may bill under FQHC PPS instead of AIR. Facilities in that category should also reference the companion FQHC brief, since some encounters route through FQHC PPS lanes.
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What Counts as a Qualifying AIR Encounter

This is the federal rule that governs every Tribal Health Program conversation about 92229. Same face-to-face logic as FQHC PPS, but routed through IHS billing rules.

An AIR-billable encounter requires a face-to-face with a qualifying provider (physician, nurse practitioner, physician assistant, certified nurse midwife, clinical psychologist, clinical social worker, or dentist for dental encounters). A stand-alone 92229 delivered by a medical assistant or ophthalmic tech without a qualifying-provider touch does not trigger an AIR payment.

What DOES qualify:

- A 92229 delivered during an existing qualifying visit (Annual Wellness Visit, diabetic chronic care management visit, problem visit). The screening is bundled into the encounter; the encounter still pays full AIR.
- A "dedicated" visit that pairs 92229 delivered with a brief qualifying-provider touch (NP, PA, MD, CNM for a diabetic status check or related care).

What does NOT qualify:

- A screening-only visit with no qualifying-provider face-to-face. AIR is not triggered.
- "Incident-to" services performed without a qualifying-provider touch.

Sources: [IHS Billing Instructions for Medicare and Medicaid, 42 CFR 405.2463](#) (FQHC parallel for context).

How the AIR Works

- **Per-encounter payment**, set annually by IHS and published in the Federal Register.
- **CY 2025 outpatient AIR (Lower 48):**
 - Medicare Part B: \$718
 - Non-Medicare (Medicaid, other): \$801
- **CY 2025 outpatient AIR (Alaska):**
 - Medicare Part B: \$1,193
 - Non-Medicare: \$1,209
- **CY 2026 outpatient AIR (Medicare, Lower 48): \$826.** Alaska rate: \$1,222. Inpatient rates differ (CY 2026 inpatient: \$5,707 Lower 48, \$5,208 Alaska, paid PPS or CAH cost-basis depending on facility).
- Medicare pays AIR for qualifying outpatient encounters at IHS-direct and Tribal 638 facilities.
- Medicaid pays AIR with **100% Federal Medical Assistance Percentage (FMAP)** for services furnished to American Indians and Alaska Natives, per 42 USC 1396d(b). States are required to pay the IHS-equivalent rate for services to AI/AN Medicaid beneficiaries. Operational mechanics are governed by State Health Official letter SHO #16-002 and per-state Medicaid agreements.
- **Urban Indian Organization (UIO) caveat:** 100% FMAP applicability for UIOs was extended through temporary federal provisions; current status depends on subsequent congressional action. UIO

facilities should confirm their 100% FMAP status with their state Medicaid program and CMS before treating it as in effect.

- **Medicare coinsurance waiver:** Medicare-eligible American Indians and Alaska Natives receiving care at IHS or Tribal 638 facilities are not billed the 20 percent coinsurance. The statutory anchor is 42 USC 1395qq, with operational rules in 42 CFR Part 136.

Is 92229 Separately Payable Under AIR?

No. The AIR is all-inclusive and bundles diagnostic imaging, lab, and other services performed on the same day as the qualifying encounter. 92229 delivered during a qualifying visit captures no additional AIR payment but does capture the full encounter rate.

Why the Tribal Lane Economics Are Different (and Often Better) Than FQHC PPS

- The CY 2026 outpatient AIR (Medicare, Lower 48) is **\$826**, materially higher than the Medicare FQHC PPS base rate of \$207.72 (boosted to \$278.68 for AWV / IPPE Initial Preventive Physical Examination new-patient visits).

The CY 2025 non-Medicare outpatient AIR (Lower 48) was \$801, with the CY 2026 non-Medicare rate

- published in the Federal Register.

Medicaid pass-through at 100% FMAP for AI/AN beneficiaries (per 42 USC 1396d(b)) means AI/AN

- Medicaid encounters route through the same elevated rate structure.

The Medicare coinsurance waiver means your Tribal facility does not need to collect the 20% patient

- coinsurance for AI/AN patients, removing a collection-cost and care-access friction point.

The combination of higher AIR + 100% FMAP pass-through + no coinsurance collection makes the per-encounter financial picture meaningfully stronger than the FQHC PPS lane.

Quality Measure Performance: GPRA and HEDIS

GPRA Diabetic Retinopathy Screening Rate

- DR screening is a Government Performance and Results Act (GPRA) Clinical Performance Measure (CPM) that Tribal and IHS facilities report against annually.
- **FY 2025 national IHS target rate: 47.6%** (FY 2024 target was 44.7%).
- Per the IHS-JVN November 2025 teleophthalmology AI handout, AI-referred patients follow up at the eye institute approximately 3 times more often than teleophthalmology-only referrals, a meaningful operational signal that autonomous AI improves the screening-to-treatment pipeline.

- Autonomous AI screening typically moves the reported GPRA rate meaningfully because it removes the referral-to-completion gap that historically pulls AI/AN patients out of the screening pipeline.

HEDIS Eye Exam for Patients with Diabetes (Where Applicable)

- For Tribal Health Programs serving members under Medicaid Managed Care or Medicare Advantage contracts, HEDIS EED is the relevant payer-facing quality measure.
 - DR screening compliance maps to MCO pay-for-performance bonuses, typically in the 1 to 2 percent of plan value range, though contract-specific terms govern.
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SDPI Grant Performance Protection

- The Special Diabetes Program for Indians (SDPI) funds Tribal diabetes care across the country.
 - **SDPI was reauthorized through December 31, 2026 at \$200 million per year (up from \$160 million; signed February 2026).**
 - DR screening (Eye Exam / Retinopathy Screening) is one of the published SDPI Best Practices.
 - Partner Tribes funded under SDPI commonly have DR screening performance built into their grant scope of work.
 - Improving DR screening rate protects and strengthens SDPI grant renewal competitiveness.
 - Sustained low screening rates can affect grant scope and renewal positioning.
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Reduced Referral Leakage

This is operationally important and often understated. AI/AN patients referred out for specialty DR screening often do not complete the referral due to:

- Geographic distance from specialty providers, particularly in rural and reservation-based settings.
- Transportation barriers.
- Specialist availability and scheduling lag.
- Cultural and trust factors that favor in-clinic care delivery.

Point-of-care autonomous AI screening closes the completion gap by delivering the screening during a visit the patient is already attending. The IHS-JVN November 2025 teleophthalmology AI handout reports that AI-referred patients complete follow-up at the eye institute at roughly 3 times the rate of teleophthalmology-only referrals. Peer-reviewed work documents lower DR screening completion rates among American Indian / Alaska Native populations relative to other groups, including Lundeen et al. (2023) and the IHS-Joslin Vision Network cohort study (n=53,998).

Summary Table for Tribal / IHS Reimbursement Lanes

REIMBURSEMENT LANE	RATE UNIT	APPROX. AMOUNT	IS 92229 SEPARATELY PAYABLE?
Medicare AIR (IHS-direct or Tribal 638), Lower 48	Per-encounter (all-inclusive)	\$718 outpatient CY 2025; \$826 CY 2026	No, bundled
Medicare AIR, Alaska	Per-encounter	\$1,193 CY 2025; \$1,222 CY 2026	No, bundled
Non-Medicare AIR (Medicaid and other), Lower 48	Per-encounter	\$801 CY 2025 outpatient	No, bundled
Medicaid AIR with 100% FMAP pass-through	Per-encounter	IHS-equivalent rate by state	No, bundled
Medicare Advantage at Tribal facility	Per-encounter (with applicable wrap)	Varies	No, bundled
Medicaid Managed Care at Tribal facility	Plan-negotiated rate, AI/AN routed through AIR	Varies	No, drives HEDIS bonus

For context only, non-Tribal Medicare FFS providers receive approximately \$40 to \$55 per 92229 screen. **This per-screen number does not map to Tribal Health Program economics.**

The Three-Part Value Story for a Tribal Health Program Deploying AEyeDS

Since 92229 does not generate a per-screen fee at a Tribal facility, the financial case should be framed in these three stacked components:

1. Encounter Capture at the AIR

- Every AI/AN diabetic patient on your panel who is currently referred out for retinal screening is an encounter your facility does not bill.
- At the CY 2026 outpatient Medicare AIR of \$826 (Lower 48), one avoided referral represents materially more revenue than the equivalent FQHC PPS encounter would (\$278.68 boosted, \$207.72 base).

- 92229 delivered in-clinic is an encounter that would otherwise go to an external ophthalmologist or retina specialist, with high no-show and incomplete-referral rates in rural and reservation-based settings.

2. GPRA DR Screening Rate Performance

- DR screening is a measured GPRA performance metric for Tribal and IHS facilities.
- Autonomous AI screening removes the referral-to-completion gap and typically moves the reported rate meaningfully.
- Sustained performance positions your facility favorably in IHS quality reporting and in any partnering MCO contracts.

3. SDPI Grant Performance Protection

- For Tribes and Tribal organizations operating SDPI-funded diabetes programs, DR screening rate is commonly a scope-of-work objective.
- Improving the rate strengthens grant renewal positioning.
- Sustained low rates can affect grant scope and competitiveness.

The AWV Pull-Through Play (the cleanest tactical path)

Given that 92229 is bundled in the AIR, and that a screening-only visit does not trigger an AIR encounter, the workflow most Tribal billing offices will sign off on is the Annual Wellness Visit (AWV) pull-through play. (For Tribal facilities that bill Medicare.)

The setup:

1. Pull a list of Medicare-aged diabetic AI/AN patients on your panel who have not had an Annual Wellness Visit in the current benefit year.
2. Proactively outreach that list for AWV scheduling.
3. Deliver 92229 during the AWV, in the same visit.

The math on a single Medicare visit (Tribal AIR example):

COMPONENT	AMOUNT
Medicare AIR outpatient (CY 2026, Lower 48)	\$826
Medicare AIR outpatient (CY 2026, Alaska)	\$1,222
92229 captured in the encounter	bundled, no incremental fee
GPRA DR screening rate moves	yes, one visit closes the patient's measure
20% Medicare coinsurance	waived for AI/AN patients at IHS or Tribal 638 facilities

Why this works:

- The AWV is a qualifying preventive service, so the encounter is AIR-billable.
- 92229 inside the AWV captures the diagnostic screening without requiring a second encounter or a dedicated DR screening day.
- GPRA DR screening rate moves on the same visit.
- For SDPI-funded facilities, screening performance is captured in grant-tracked metrics.

Why it beats the "dedicated DR screening visit" framing:

- Dedicated DR screening days without a qualifying provider touch do not trigger an AIR encounter.
- Dedicated screening days with a qualifying provider touch do work, but they consume scarce NP/PA time per patient and compete with existing visit templates.
- AWV pull-through uses a visit slot you are already running, adds 92229 to it, and closes both the GPRA measure and any applicable HEDIS measure in the same encounter.

What your Tribal billing office should confirm:

- AWV eligibility for each targeted diabetic patient (once per benefit year, not within 12 months of an IPPE).
- IHS Area Office guidance on documenting 92229 as an incident diagnostic during the AWV encounter at AIR.
- For SDPI-funded facilities: whether the SDPI grant scope captures the autonomous AI screening as a contributing intervention to the DR screening rate objective.
- 100% FMAP pass-through handling for AI/AN Medicaid beneficiaries by your state Medicaid program.

Sources: [IHS Reimbursement Rates](#), [Federal Register annual publication](#), [IHS Billing Instructions for Medicare and Medicaid](#), [IHS Special Diabetes Program for Indians \(SDPI\)](#), [IHS GPRA Performance Measures](#).

Equipment Considerations Specific to Tribal Settings

Two operational notes for Tribal deployment that frequently come up in scoping conversations:

1. **Handheld vs. tabletop cameras.** Tribal Health Programs serving multiple satellite clinics, mobile units, or community outreach sites benefit from handheld cameras that can move between sites. AEYE-DS is FDA-cleared with both the Optomed Aurora (handheld) and the Topcon NW400 (tabletop). For comparison: the other FDA-cleared autonomous AI DR screening products are currently cleared for tabletop cameras only (Digital Diagnostics LumineticsCore with Topcon TRC-NW400; Eyenuk EyeArt with Canon CR-2 AF, Canon CR-2 Plus AF, or Topcon NW400). The right form factor depends on your service-delivery model.
2. **Existing diabetes program integration.** Many Tribal Health Programs already have diabetes navigators or community health representatives doing outreach for diabetic foot exams, A1C testing, and lab work. Adding 92229 into the existing diabetes navigator workflow is operationally lighter than building a new DR screening clinic from scratch.

What Your Tribal Health Program Should Confirm Before Deploying

1. **Current CY 2026 IHS outpatient AIR** for your IHS Area. National rates are published in the Federal Register annually (CY 2026: \$826 Lower 48 Medicare, \$1,222 Alaska Medicare). Confirm any Area-specific rate adjustment with your IHS Area Office.
 2. **Billing status** (IHS-direct vs. Tribal 638 vs. Urban Indian Organization vs. FQHC-Look-Alike). These have different billing mechanics. UIOs in particular should confirm current 100% FMAP applicability with their state Medicaid program before treating it as in effect.
 3. **100% FMAP pass-through treatment** for AI/AN Medicaid beneficiaries by your state Medicaid program. Operational mechanics are governed by SHO #16-002 and state-by-state Medicaid agreements.
 4. **GPRA reporting alignment** so autonomous AI screening counts toward the reported DR screening rate (FY 2025 national target: 47.6%).
 5. **SDPI grant scope of work** if applicable, specifically whether DR screening rate is a measured objective. SDPI is reauthorized through Dec 31, 2026 at \$200M per year.
 6. **Equipment fit** (handheld vs. tabletop) based on your service-delivery footprint (single clinic vs. multi-site vs. mobile).
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Next Steps

If your Tribal Health Program wants to evaluate AEyeDS deployment:

- Contact Proactive FQHC at grants@proactivefqhc.org for partnership and grant-program guidance.
 - Contact AEYE Health at mike@aeyehealth.com (Regional Sales Manager, West) for product, deployment, and pilot scoping.
 - The AEYE ROI calculator at <https://aeyehealthroi.netlify.app/> is available for initial financial modeling, with a more detailed version linked from Proactive's retinopathy resources page.
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Source List

- [IHS Reimbursement Rates, Federal Register annual publication](#)
 - [Federal Register CY 2025 IHS Reimbursement Rates](#)
 - [Federal Register CY 2026 IHS Reimbursement Rates](#)
 - [IHS Billing Instructions for Medicare and Medicaid](#)
 - [42 USC 1396d\(b\), 100% FMAP for IHS services](#)
 - [42 USC 1395qq, Medicare payments to IHS facilities](#)
 - [IHS Special Diabetes Program for Indians \(SDPI\)](#)
 - [IHS SDPI Best Practices: Eye Exam / Retinopathy Screening](#)
 - [Senator Collins SDPI \\$200M Reauthorization Announcement](#)
 - [IHS GPRA Clinical Performance Measure Matrix FY 2023-2025](#)
 - [IHS-JVN November 2025 Teleophthalmology AI Handout](#)
 - [42 CFR 405.2463, qualifying visit definition \(FQHC parallel\)](#)
 - [Noridian FQHC Medicare Advantage Wrap-Around Payment \(parallel mechanics\)](#)
 - [AEYE Health FAQ \(handheld and tabletop camera options\)](#)
 - [AEYE Health: Billing and Reimbursement for Autonomous Diabetic Eye Exam](#)
 - [Digital Diagnostics: CMS Finalizes National Rate for CPT 92229](#)
 - [Lundeen et al., DR screening disparities \(PMC9999279\)](#)
 - [IHS-Joslin Vision Network teleophthalmology cohort study \(PMC6010214\)](#)
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Note: Version 2b - Prepared by AEYE Health in partnership with Proactive FQHC. This brief is directional planning information only, not a quote, not a contract. Confirm all figures with your MAC, IHS Area Office, state Medicaid program, and MCO contract administrator.